2024, Mar. 2024 Release LOC:Acute Adult
Dehydration or Gastroenteritis

Utilization Benchmarks: *Multiple options available, see table below for options*

Overview

Instruction:
This subset is for the management of dehydration or uncomplicated infectious gastroenteritis and excludes hemolytic uremic syndrome. For electrolyte abnormalities or clinical findings that do not meet criteria in dehydration or gastroenteritis, refer to the appropriate condition specific subset. Causes of dehydration commonly include vomiting, diarrhea, inadequate access or intake of water, excessive perspiration, or increased urination (Lazarciuc, In: Rosen's Emergency Medicine: Concepts and Clinical Practice, 9 edn. 2018: 249-56.e2).

Introduction:
Gastroenteritis is an inflammation of the intestines that causes digestive upset. Symptoms of gastroenteritis typically include mild diarrhea, abdominal pain and cramps, low-grade fever, headache, nausea, or vomiting, with or without the presence of mucous or blood in the stool. In older patients or people with weakened immune defenses, gastroenteritis can lead to severe dehydration, electrolyte imbalances, and hypovolemia which may require hospitalization. Symptoms of dehydration may include (Tomas Nguyen and Saadia Akhtar, In: Rosen's Emergency Medicine, 9 edn. 2018:1129-49.e2):

- Increased thirst
- Diminished urine output
- Dry skin or mouth
- Absent tears
- Headache
- Dizziness

Evaluation and Treatment:
In most cases, dehydration or gastroenteritis is diagnosed based on symptoms, history of exposure to spoiled food, impure water, or someone with diarrhea, excessive fluid loss, or lack of access or intake of water, and by physical examination. Severity of dehydration should be determined by assessing the following: weight loss, general appearance, mental status, urine output, peripheral perfusion, vital signs, and skin turgor. Mild dehydration can be managed in the home or community setting with oral replacement therapy (ORT). Adults requiring hospital admission for severe dehydration will require laboratory evaluation of sodium, potassium, blood urea nitrogen (BUN), creatinine, and glucose concentrations. Intravenous (IV) fluids should be considered when the patient is moderately to severely dehydrated, unable to tolerate ORT, has altered mental status, difficulty breathing, or is exhibiting signs of hypovolemic

shock (Tomas Nguyen and Saadia Akhtar, In: Rosen's Emergency Medicine, 9 edn. 2018:1129-49.e2).

InterQual® criteria are derived from the systematic, continuous review and critical appraisal of the most current evidence-based literature and include input from our independent panel of clinical experts. To generate the most appropriate recommendations, a comprehensive literature review of the clinical evidence was conducted. Sources searched included PubMed, Agency for Healthcare Research and Quality (AHRQ) Comparative Effectiveness Reviews, the National Institute for Health and Care Excellence (NICE), the Centers for Medicare and Medicaid Services (CMS), The Cochrane Library, and The Joint Commission. Other medical literature databases, medical content providers, data sources, regulatory body websites, and specialty society resources may also have been utilized. Relevant studies were assessed for risk of bias following principles described in the *Cochrane Handbook*. The resulting evidence was assessed for consistency, directness, precision, effect size, and publication bias. Observational trials were also evaluated for the presence of a dose-response gradient and the likely effect of plausible confounders. The content is based on a variety of references which are cited at specific criteria points throughout the subset.

(Excludes PO medications unless noted)
Select Day, **One:**
- **Initial review, One:** [1, 2]
  (From arrival in ED through decision to admit)
  (Symptom or finding within 24h)
  - **OBSERVATION, Both:** [3]
    - Dehydration or gastroenteritis, ≥ **One:**
      - At least 1L IVF given prior to decision to admit and, ≥ **One:**
        - Heart rate, sustained > 100/min [4]
        - Mental status changes (excludes coma, stupor, or obtundation) [5]
        - Orthostatic hypotension, **Both:** [6, 7]
          - Sustained drop in blood pressure within 3 min of sitting or standing
          - Systolic BP drop ≥ 20 mmHg or diastolic BP drop ≥ 10 mmHg
      - Sodium > 150 mEq/L(150 mmol/L) [8]
      - Persistent vomiting after 2 doses of antiemetic [9]
    - IV fluid resuscitation [10]
- **Episode Day 1, One:** [1]
  (Symptom or finding within 24h)
  - **OBSERVATION, Both:** [3]
    - Dehydration or gastroenteritis, ≥ **One:**
      - At least 1L IVF given prior to decision to admit and, ≥ **One:**
        - Heart rate, sustained > 100/min [4]
        - Mental status changes (excludes coma, stupor, or obtundation) [5]
        - Orthostatic hypotension, **Both:** [6, 7]
          - Sustained drop in blood pressure within 3 min of sitting or standing
          - Systolic BP drop ≥ 20 mmHg or diastolic BP drop ≥ 10 mmHg
      - Sodium > 150 mEq/L(150 mmol/L) [8]
      - Persistent vomiting after 2 doses of antiemetic [9]
    - IV fluid resuscitation [10]
- **Episode Day 2, One:** [11]
  - **OBSERVATION, One:**
    - **Responder**, discharge expected today if clinically stable last 12h, **All:** [12]
      - Tolerating PO [13]
      - Vomiting or diarrhea improving
      - Laboratory values within acceptable limits [14]
      - Mental status within normal limits or returned to baseline
      - Vital signs within acceptable limits [15]
    - **Partial responder**, not clinically stable for discharge and requires continued stay, **Both:**
      - Finding, ≥ **One:**
        - Heart rate, sustained > 100/min [4]
        - Mental status changes (excludes coma, stupor, or obtundation) [5]
        - Orthostatic hypotension, **Both:** [6, 7]
          - Sustained drop in blood pressure within 3 min of sitting or standing
          - Systolic BP drop ≥ 20 mmHg or diastolic BP drop ≥ 10 mmHg
        - Sodium > 150 mEq/L(150 mmol/L) [8]
        - Potassium < 3.0 mEq/L(3.0 mmol/L) and no ECG changes [16, 17, 18]
        - Vomiting, persistent [9]
      - Intervention, ≥ **One:** [19]
        - Antiemetic ≥ 3x/24h [20]
        - IV fluid [21]
        - Serotonin antagonist ≥ 2x/24h [22]
        - Potassium repletion (includes PO)
- **Episode Day 3, One:** [23]

- **OBSERVATION, One:** (3)
  - **Responder**, discharge expected today if clinically stable last 12h, **All:** (12)
    - Tolerating PO (13)
    - Vomiting or diarrhea improving
    - Complication or comorbidity, ≥ **One:**
      - No complication or active comorbidity relevant to this episode of care (24)
      - Laboratory values within acceptable limits (14)
      - Mental status within normal limits or returned to baseline
      - Vital signs within acceptable limits (15)
  - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, **One:**
    - For dehydration or gastroenteritis use Episode Day 3 at a higher level of care
    - For a condition other than dehydration or gastroenteritis use appropriate subset (Episode Day 1) based on clinical findings
    - Refer for secondary review
- **ACUTE, Both:**
  - Finding, ≥ **One:** (25)
    - Mental status changes (excludes coma, stupor, or obtundation) or Glasgow Coma Scale 9-14 (5)
    - Orthostatic hypotension, **Both:** (6, 7)
      - Sustained drop in blood pressure within 3 min of sitting or standing
      - Systolic BP drop ≥ 20 mmHg or diastolic BP drop ≥ 10 mmHg
    - Sodium > 150 mEq/L(150 mmol/L) (8)
    - Potassium < 3.0 mEq/L(3.0 mmol/L) and no ECG changes (16, 17, 18)
    - Vomiting, persistent (9)
  - Intervention, ≥ **One:** (19)
    - Antiemetic ≥ 3x/24h (20)
    - IV fluid (21)
    - Serotonin antagonist ≥ 2x/24h (22)
    - Potassium repletion (includes PO)

- **Episode Day 4, One:**
  - **ACUTE, One:**
    - **Responder**, discharge expected today if clinically stable last 12h, **All:** (12)
      - Tolerating PO (13)
      - Vital signs within acceptable limits (15)
      - Complication or comorbidity, **One:**
        - No complication or active comorbidity relevant to this episode of care (26)
        - Complication or comorbidity and clinically stable for discharge, ≥ **One:**
          - Vomiting or diarrhea improving
          - Laboratory values within acceptable limits (14)
          - Mental status or GCS within normal limits or returned to baseline
    - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, **One:**
      - **Use Extended Stay subset**
      - Refer for secondary review

Discharge, Level of Care, DISCHARGE SCREENS, One: (27)

- **HOME, Both:**
  - Level of care appropriateness, **Both:**
    - Home environment safe and accessible (28)
    - Patient or caregiver demonstrates ability to manage care
  - Dehydration or gastroenteritis discharge planning, **All:**
    - Provide comprehensive written discharge and teaching instructions (29)
    - Dehydration or gastroenteritis discharge education

      - Medication reconciliation [30]
      - Follow-up care planned [31]
      - Identify and address transportation needs
- **HOME CARE, Both:**
  - Level of care appropriateness, **All:**
    - Home environment safe and accessible [28]
    - Patient or caregiver willing and able to learn care
    - Outpatient management contraindicated or unavailable [32, 33]
    - Skilled services, ≥ **One:** [34]
      - Skilled nursing [35]
      - Skilled therapy, PT, OT, or ST
      - Behavioral health [36]
  - Dehydration or gastroenteritis discharge planning, **All:**
    - Provide comprehensive written discharge and teaching instructions [29]
    - Dehydration or gastroenteritis discharge education
    - Medication reconciliation [30]
    - Follow-up care planned [31]
    - Identify and address transportation needs
- **BEHAVIORAL HEALTH, Both:**
  - Level of care appropriateness, **One:**
    - Support systems available [37]
  - Skilled treatment, ≥ **One:**
    - Clinical assessment [38]
    - Individual, group, or family counseling
    - Psychiatric, substance, or medication evaluation [39]
- **SKILLED MEDICAL OR THERAPY, Both:**
  - Level of care appropriateness, **All:**
    - Medical practitioner, NP, PA assessment or oversight ≥ 1x/wk
    - Treatment precluded in a lower level
    - Services, ≥ **One:**
      - Medical and skilled nursing interventions or assessment 1-2x/24h [35]
      - Therapy, **All:**
        - Goal directed therapy with rehabilitation potential [40, 41]
        - Functional impairments requiring at least supervision [42]
        - Able to tolerate 1h/d of skilled therapy ≥ 5d/wk
  - Complete prior to facility transfer, **All:**
    - Medication reconciliation [30]
    - Obtain and complete forms for facility
    - Obtain discharge summary and transmit to facility and medical practitioner
    - Arrange transportation
- **SUBACUTE MEDICAL OR THERAPY, Both:**
  - Level of care appropriateness, **All:**
    - Medical practitioner, NP, PA assessment or oversight ≥ 2x/wk
    - Treatment precluded in a lower level
    - Services, ≥ **One:**
      - Medical and skilled nursing interventions or assessment 4h/24h [35]
      - Therapy, **All:**
        - Goal directed therapy with rehabilitation potential [40, 41]
        - ≥ 2 functional impairments requiring at least minimum assistance [42]
        - Able to tolerate 2-3h/d of skilled therapy ≥ 5d/wk and ≥ 1 therapy discipline
  - Complete prior to facility transfer, **All:**
    - Medication reconciliation [30]
    - Obtain and complete forms for facility

  - Obtain discharge summary and transmit to facility and medical practitioner
  - Arrange transportation
- **SUBACUTE REHAB, Both:**
  - Level of care appropriateness, **All:**
    - Medical practitioner, NP, PA assessment or oversight ≥ 3x/wk
    - Treatment precluded in a lower level
    - Services, **All:**
      - Goal directed therapy with rehabilitation potential [40, 41]
      - ≥ 2 functional impairments requiring at least minimum assistance [42]
      - Able to tolerate 2-3h/d of skilled therapy ≥ 5d/wk and ≥ 2 therapy disciplines
  - Complete prior to facility transfer, **All:**
    - Medication reconciliation [30]
    - Obtain and complete forms for facility
    - Obtain discharge summary and transmit to facility and medical practitioner
    - Arrange transportation
- **ACUTE REHAB, Both:**
  - Level of care appropriateness, **Both:**
    - Medical practitioner, NP, PA assessment or oversight ≥ 3x/wk
    - Services, **All:**
      - Goal directed therapy with rehabilitation potential [40, 41]
      - ≥ 2 functional impairments requiring at least minimum assistance [42]
      - Able to tolerate ≥ 15h/wk of skilled therapy and ≥ 2 therapy disciplines [43]
  - Complete prior to facility transfer, **All:**
    - Medication reconciliation [30]
    - Obtain and complete forms for facility
    - Obtain discharge summary and transmit to facility and medical practitioner
    - Arrange transportation
- **LONG TERM ACUTE CARE, Both:**
  - Level of care appropriateness, **Both:**
    - Treatment precluded in a lower level
    - In lieu of acute or continued hospitalization or failed lower level of care, ≥ **One:**
      - Primary condition or illness and treatment of active comorbid condition(s) [44]
      - Ventilator dependent ≥ 6h/d and weaning planned [45, 46]
      - Acute and comorbid conditions requiring prolonged hospitalization [47]
  - Complete prior to facility transfer, **All:**
    - Medication reconciliation [30]
    - Obtain and complete forms for facility
    - Obtain discharge summary and transmit to facility and medical practitioner
    - Arrange transportation

Utilization Benchmarks

Condition or Procedure	% Pd Obs	LOS (days)	Type
391 ESOPHAGITIS, GASTROENTERITIS AND MISCELLANEOUS DIGESTIVE DISORDERS WITH MCC	**n/a**	**3.9**	**CMS GMLOS**
392 ESOPHAGITIS, GASTROENTERITIS AND MISCELLANEOUS DIGESTIVE DISORDERS WITHOUT MCC	**n/a**	**2.6**	**CMS GMLOS**
640 MISCELLANEOUS DISORDERS OF NUTRITION, METABOLISM, FLUIDS AND ELECTROLYTES WITH MCC	**n/a**	**3.6**	**CMS GMLOS**

Condition or Procedure	% Pd Obs	LOS (days)	Type
641 MISCELLANEOUS DISORDERS OF NUTRITION, METABOLISM, FLUIDS AND ELECTROLYTES WITHOUT MCC	**n/a**	**2.6**	**CMS GMLOS**
DEHYDRATION	**52%**	**3.1**	**InterQual**
GASTROENTERITIS	**30%**	**2.9**	**InterQual**

Percent Paid as Observation indicates the final payment status as a percentage for patients hospitalized with the condition. It may not correspond to the status assigned at admission. Note that patients initially placed in Observation status who are converted to inpatient status (common) are included in the inpatient category, and that patients initially admitted to inpatient status who are converted to observation status (uncommon) are included in the Observation category.

Notes:

1:

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital-level services **AND** that the patient has a hospital stay of at least two midnights. Short-stay exceptions to this rule are defined below. Application of InterQual® criteria can help an organization comply with the medical necessity requirements of the two-midnight rule.
For those who are impacted by the CMS Two-Midnight Rule, follow the steps below:

1. **Determine the need for hospital-based services by applying criteria.** If a patient meets criteria at any level of care, they have met the medical necessity requirement of the rule.
2. **Determine whether the patient is an inpatient or assign observation status** depending on which level of care is met and the expected length of stay. Based on the presentation of the patient, evidence-based criteria such as InterQual® can support the provider's expectation for length of stay.

- **When care is expected to span two midnights** and the patient has met criteria at the Acute, Intermediate or Critical level of care, an inpatient status may be appropriate.

NOTE: In the event of an "Unforeseen Circumstance" per CMS resulting in a shorter stay than initially expected, a status of inpatient may still be appropriate:

- Leaving against medical advice (AMA)
- Unexpected death
- Transfer of the patient
- Unexpected clinical improvement
- Election of hospice care

- **When care is expected to span less than two midnights** but Acute, Intermediate, or Critical criteria are met, it may be appropriate to initially assign a status of observation.

NOTE: CMS has defined the following exceptions as being appropriate for inpatient status regardless of length of stay:

- Mechanical Ventilation, established during current hospitalization
- Surgical procedure or intervention found on the CMS Inpatient Only List

- **When it is undecided as to whether care will span two midnights**, the reviewer may use the level of care met as a guide to determine whether the patient is appropriate as an inpatient or observation status. The criteria consider multiple factors, including clinical severity, comorbidity, and risk, when differentiating between the Observation and Acute levels of care. These factors impact the length of stay. A patient meeting criteria at the Acute levels of care can generally be expected to have a length of stay of greater than two midnights and thus could be assigned to inpatient status. Patients who meet criteria at the Observation level of care are more appropriate for observation status.

(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

2:

Initial Review criteria are a level of care determination tool, intended to be used as **real-time** decision support in the emergency department to identify if observation or inpatient hospital level services are warranted. They help the reviewer determine whether a patient is appropriate for observation or inpatient admission at the time a decision to admit the patient is being made. Initial Review criteria evaluate **only data that are available at the time the decision is being made**. This may include previously provided interventions or the results of laboratory, imaging, and other tests. Initial Review may be appropriate when bridge or holding orders (e.g., admit to telemetry) are in place. These orders are intended to address the patient's needs until full treatment and medication orders are written.
Initial Review criteria **should not be used** retrospectively or once the decision to admit has been made (e.g., when full treatment and medication orders have been written). If the reviewer has sufficient information to complete an Episode Day 1 review, an Initial Review need not be completed.
Note: While Initial Review enables identification of the level of care, it is not intended to, and should not be used as a substitute for an Episode Day 1 review, which will generally include specific, evidence-based interventions and

intensity of service requirements.
For further clarification, see the Acute Level of Care Review Process.

3:

Hemodynamically stable patients who require at least 6 hours, and for certain conditions, up to 48 hours, of treatment or assessment pending a decision regarding the need for additional care. Observation level of care services may be provided in designated observation units or on a hospital floor.
Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions consider a variety of factors, such as clinical judgment, medical necessity, payer policies and contracts, and state and national regulations.

4:

Vital signs are considered sustained findings when they are abnormal for 2 or more readings greater than 15 minutes apart or are lasting at least 15 minutes. This excludes an isolated reading, a transient abnormal measurement, or a finding that requires urgent treatment (e.g., severe hypoxemia, ventricular arrhythmia, hypotension). Application of criteria for a vital sign finding should be based on confirmatory measurements repeated at regular intervals. Events that are infrequent or vital sign abnormalities that have resolved with outpatient treatment are not considered to be sustained (e.g., resolution of hypertension following anti-hypertensive therapy, resolution of tachycardia following rehydration).
Note: The definition of sustained reflects the opinion of InterQual's® external peer reviewers. This criteria point is based upon current best practice and is the product of an iterative process involving multiple clinicians with diverse expertise in varied geographic settings.

5:

Mental status change may include confusion, disorientation, delirium, or increasing lethargy (Lacey et al., Ann Med 2019, 51: 232-51).
Instruction: Patients with acute coma, stupor, or obtundation should be reviewed at a higher level of care due to the need for more frequent neurological monitoring. These criteria exclude chronic coma, stupor, or obtundation (Shenvi et al., Ann Emerg Med 2020, 75: 136-45).

6:

Tachycardia and orthostatic hypotension are common symptoms of dehydration when vomiting or diarrhea are present (Tomas Nguyen and Saadia Akhtar, In: Rosen's Emergency Medicine, 9 edn. 2018:1129-49.e2).

7:

Orthostatic hypotension (Juraschek et al., JAMA Intern Med 2017, 177: 1316-23).

8:

Hypernatremia and dehydration (Camiron L. Pfennig and Corey M. Slovis, In: Rosen's Emergency Medicine. 2018: 1516-32.e2)

9:

Patients with mild to moderate active vomiting should be treated with intravenous hydration and antiemetic therapy. A dehydrated patient who cannot tolerate oral hydration should not be discharged (Joshua Guttman, In: Rosen's Emergency Medicine, 9 edn. 2018:230-41.e1).

10:

Fluid resuscitation in severe dehydration involves the administration of repeated intravenous fluid boluses to acutely restore depleted intravascular volume. The administration of 20 mL/kg boluses until adequate urine output, resolution of symptoms, or adequate hydration without further losses is recommended (Piescik-Lech et al., Aliment Pharmacol Ther 2013, 37: 289-303).
Patients who received IV fluid totaling 2.5 L per day or more require close electrolyte monitoring as this amount can cause significant hyponatremia. For patients with decreased cardiac reserve, significant renal dysfunction, or

third-spacing, smaller volume boluses may be used. For patients with obesity, adjust IV fluid rates to their ideal body weight (National Institute for Health and Care Excellence, Intravenous fluid therapy in adults in hospital. 2017).

11:

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital level services **AND** that the patient has a hospital stay of at least two midnights.
For patients who are in observation status and do not meet Observation responder criteria on Episode Day 2, apply Episode Day 2 criteria at the Observation, Acute, Intermediate or Critical level of care in the appropriate subset to demonstrate the continued need for hospital-based services.
NOTE: For review of a surgical patient, Post-Operative Day 1 equates to Episode Day 2.
(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

12:

Selection of this criteria point indicates that the patient is responding to treatment and is clinically stable for transfer or discharge. To determine the most appropriate post-acute level of care, see discharge criteria.

13:

Oral intake should be at least equivalent to the patient's maintenance fluid requirements or within parameters which the medical practitioner deems appropriate. Maintenance fluid requirements should be increased in the presence of dehydration or insensible loss. Maintenance fluid rates are based on ideal body weight and reflect the minimum amount of fluid required to maintain hydration. Rates may be decreased in older patients or those with a history of renal impairment or heart failure (Scales, Br J Nurs 2014, 23).

14:

Lab values include:

- Electrolytes
- Blood urea nitrogen (BUN)

15:

When a criteria point states "within acceptable limits," it refers to either the patient's normal baseline, a newly established baseline, or parameters that the medical practitioner determines are acceptable.

16:

Hypokalemia rarely occurs in isolation and is usually secondary to a disease process or overuse of diuretics. Criteria should be reviewed for the underlying cause of the hypokalemia (e.g., diabetic ketoacidosis, gastroenteritis) to ensure the patient is managed appropriately.

17:

When a patient with hypokalemia is not responding to potassium repletion, the magnesium level should be checked since hypomagnesemia could be contributing to the refractory hypokalemia.

18:

In patients with no underlying heart disease, occasional, nonconsecutive premature ventricular contractions (PVCs) less than 6/min are common and not considered a concerning finding. Three or more consecutive PVCs at a rate of 100 bpm or more that terminate spontaneously within 30 seconds are considered nonsustained ventricular tachycardia (Marine, Card Electrophysiol Clin 2016, 8: 525-43; Buxton et al., Circulation 2006, 114: 2534-70).

19:

Advancing a patient's diet should occur when the patient shows signs of clinical readiness. These signs may include:

a substantial improvement in pain and reduction in diarrhea, a resolution of fever if it was present at admission, absence of nausea or vomiting, and evidence of an appetite.
Advancing the diet begins with elemental feedings or clear liquids. Once tolerated, these patients progress to full liquids and ultimately to a regular diet. With the initiation of a regular diet, the patient may resume their oral medications.

20:

Antiemetics are commonly used to subside vomiting with few adverse effects (Hesketh et al., J Clin Oncol 2017, 35: 3240-61).

21:

Instruction: This line of criteria is intended for the patient receiving condition-directed maintenance IV fluid and excludes the patient who has IV fluid ordered to keep vein open (KVO).

22:

Serotonin antagonists, such as ondansetron are commonly used to subside vomiting with few adverse effects (Hesketh et al., J Clin Oncol 2017, 35: 3240-61).

23:

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital level services **AND** that the patient has a hospital stay of at least two midnights.
For patients who are in observation status beyond the second midnight (Episode Day 3) and who do not meet Observation responder criteria, apply Episode Day 3 Inpatient criteria in the appropriate condition-specific or general subset.
Note: Patients who meet criteria **at any of** the Inpatient levels of care **based on medically necessary hospital-based services and supported by clinical documentation would be appropriate for inpatient status;** those who do not meet criteria would be appropriate for secondary review.
(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

24:

Relevant complications or comorbidities are conditions that require active management during an inpatient stay.

25:

Patients that do not improve within the first 48h, are immunocompromised or of advanced age and may require hospitalization for further management of dehydration (Tomas Nguyen and Saadia Akhtar, In: Rosen's Emergency Medicine, 9 edn. 2018:1129-49.e2).

26:

Relevant complications or comorbidities are conditions that actively require management during an inpatient stay.

27:

The discharge screen is a resource tool and not criteria. Referring to the discharge screen at the initiation of discharge planning is recommended.

28:

The home environment and safety assessment will normally include an evaluation of the patient's pre- and post-hospitalization functional level, physical layout of the home (e.g., entrances and exits, stairs, access to the community), identification of unsafe conditions (e.g., scatter rugs, missing handrails, oxygen use and smoking, lack of fire safety devices, inadequate lighting, heating, and cooling), factors that may trigger symptoms (e.g., secondhand smoke, poor food choices, dysfunctional family dynamics), sanitation hazards (e.g., lack of electricity, running water, refrigeration, inadequate toilet facilities, presence of insects or rodents), and the need for adaptive equipment (e.g., walker, handrails for the tub or shower, elevated toilet seat).

29:

Ensuring the patient and/or caregiver understands all aspects of the condition and can assume responsibility for self-care is crucial in preventing readmission. Assessing a patient and/or caregiver's level of understanding after education has been provided can be difficult. The teach-back method is an effective way of providing education at the appropriate level and can also be used to assess the learner's comprehension. To use the teach-back method, discuss key points in common terms and avoid using medical jargon or unfamiliar terms. After the education has been delivered, ask the learner to repeat what was learned. Gaps or misinterpretations in the learner's explanation will pinpoint areas where communication may have failed and provide opportunity for clarification.

30:

Medication reconciliation is a formal process or technique used by health care providers and pharmacists to identify the most complete and accurate list of all medications a patient is taking at times of transitions in care (e.g., upon hospital admission, transfer from one unit to another during hospitalization, or discharge from the hospital to home or another facility). The goals of this process are to ensure medication and dosages are appropriate for the patient, resolve discrepancies in drug regimens, and ultimately prevent medication errors and reduce adverse drug events. Medication reconciliation is a Joint Commission National Patient Safety goal. Coordinating information when a patient is transferred to another setting, service, practitioner, or level of care ensures accurate medications are listed. The process is comprised of the following (Hospital: National Patient Safety Goals. 2020):

- Obtain an external list of medications (e.g., medications taken prior to admission)
- Develop a list of current medications and add them to the medical record
- Compare the medications from the external list to the current list
- Clarify inconsistencies/discrepancies (e.g., omissions, duplications, contraindications, unclear information, and changes)
- Develop a list of medications to be prescribed at discharge or transfer
- Communicate the new list to the patient and/or appropriate caregiver(s)
- Ensure that patient and/or caregiver(s) understand the medication information upon discharge

Specific medication issues identified as being problematic include missing medication information from transfer orders, lack of information on medications provided in the acute setting, incomplete medication records, discrepancies between hospital regimen and discharge summary, and missing information pertaining to the patient's tolerance of a medication regimen. Although medication reconciliation is an important aspect in patient safety, there is a lack of consensus and evidence about the best effective methods of implementing this process. Physician-led and electronic medication reconciliation in hospitals are effective strategies to reduce medication discrepancies, however, the impact of these interventions is uncertain due to the low quality of evidence (Choi and Kim, J Clin Pharm Ther 2019, 44: 932-45; Redmond et al., Cochrane Database Syst Rev 2018, 8: CD010791). Trained pharmacy technicians, under the direction of a licensed pharmacist, may be an option for developing and expanding medication reconciliation processes (Irwin et al., Hosp Pharm 2017, 52: 44-53).

31:

Close follow-up of older adults, pediatric, or at-risk patients after discharge can help minimize hospital readmission and total health care costs (Rising et al., Ann Emerg Med 2013, 62: 145-50; Frei-Jones et al., Pediatr Blood Cancer 2009; 52(4): 481-485; Kripalani et al., JAMA 2007; 297(8): 831-841). Studies have shown that for greater than 50% (0.50) of patients re-hospitalized within 30 days of discharge, there was no visit to a physician's office between the time of discharge and re-hospitalization (Hernandez et al., JAMA 2010, 303: 1716-22; Jencks et al., N Engl J Med 2009; 360(14): 1418-1428). Efforts to reduce readmissions should focus on patients known to be at high risk and patients who have frequent visits to the emergency department (ED). Patients at high risk may have multiple ED visits prior to readmission (Rising et al., Ann Emerg Med 2013, 62: 145-50).

32:

Outpatient management contraindicated refers to the following:

- The patient whose medical condition is so fragile that going to the clinic or medical practitioner's office for treatment would put the patient at risk
- The patient with extensive equipment (e.g., ventilators, specialized beds) where travel outside the home would be cumbersome and places the patient at risk
- The treatment goal is to adapt the home environment for increased independence (e.g., an individual

who is blind who needs mobility education)

33:

Outpatient management unavailable refers to the following:

- The patient whose treatment (e.g., dressing changes) is so extensive that it takes too long or is too frequent (e.g., twice daily) to be done in an outpatient clinic or medical practitioner's office
- The scheduling of the treatment falls outside the medical practitioner's office or clinic hours
- Travel distance to receive care is excessive (e.g., greater than 1 hour)
- The service is unavailable in the outpatient setting

34:

Skilled nursing services refer to services that must be provided by a licensed nurse qualified to assess and monitor patient condition(s) and provide medical treatment and/or teaching to patients who have skilled nursing needs. Examples of Home Care level of care interventions include skilled assessment, intervention, or education for any of the following:

- New treatment or medication regimen
- Adjustment in the treatment or medication regimen
- Infusion therapy administration, teaching, or access device management
- Management and evaluation of a care plan for patients with an active comorbidity and multiple care needs
- Chronic disease requiring a disease management program (e.g., asthma, heart failure, chronic obstructive pulmonary disease, diabetes)
- End-stage disease, hospice, or palliative care

35:

Skilled nursing interventions refer to services that are provided or supervised by a qualified or licensed professional when the care needs of the patient require assessment, observation, monitoring, and/or education to achieve the medically desired result. Close observation and assessment by nursing may be necessary at this level of care. This includes nursing care such as medication administration (excludes routine), vital signs monitoring, and/or other interventions not listed.

36:

The Home Care criteria are used for a patient who has been admitted or is expected to be admitted for home care. When criteria are met for this level of care and it is not available in your area, we recommend you use the next higher level of care.

Home care can include specialty services provided by a psychiatric nurse following a specialized treatment plan developed by a psychiatrist or advanced practice registered nurse. It is intended for treatment and monitoring of a patient's psychiatric condition that prevents the patient from leaving the house or makes leaving the house extremely difficult or unsafe (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022). Home care can also be used for a patient with severe and persistent mental illness with a history of nonadherence to medication.

Evaluation and Treatment

Programming may differ based upon legislative and geographical variances and is subject to organizational policy; however, at a minimum, it should include:

- Care coordination with treating physician and other providers
- Clinical assessment every visit
- Education every visit
- Medication assessment, teaching, and management every visit
- Medication reconciliation initiated within first visit
- Psychosocial assessment within first visit
- Substance evaluation within first 2 visits

For those patients who are covered under a Medicare benefit, the Affordable Care Act mandates that a certifying physician or allowed practitioner have a face-to-face visit with the patient within 90 days prior to the start of care, or within 30 days after the start of care. There must be documentation of the patient's condition that supports the need for home care services and the requirement of homebound status (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022).

37:

Support system includes friends, family, social services, health care delivery providers or agencies, sponsors, clergy, and self-help groups.

38:

The initial clinical assessment is performed by a licensed professional and includes a comprehensive review of the patient's presenting diagnosis and a review of body systems. Identification of current and potential medical needs and health problems can be identified by the clinical assessment. The clinical assessment includes:

- History and physical exam (e.g., vital signs, height, weight)
- Pain assessment includes cause, intensity, quality, onset, duration, and effects on quality of life (e.g., activities of daily living (ADLs), instrumental activities of daily living (IADLs), and interpersonal relationships). Pain relievers should also be included
- Nutritional and hydration status
- Functional ability
- Safety and infection control measures
- Prescribed and over-the-counter (OTC) medications, herbal supplements, and home remedies
- Patient's and/or caregiver's understanding of medication use, dosing, side effects, and adherence to the medication or treatment regimen
- Patient's and/or caregiver's understanding of the illness or disease process and potential long-term complications
- Patient's and/or caregiver's coping strategies to deal with the illness or injury and ability to follow the plan of care

39:

For specific level of care recommendations for the treatment of psychiatric and substance use disorders, refer to the InterQual® Adult and Geriatric Psychiatry, InterQual® Child and Adolescent Psychiatry, or InterQual® Substance Use Disorders products.

40:

Goal-directed therapy includes one or more of the following:

- Activities of daily living (ADLs) (e.g., feeding, dressing, bathing, grooming, toileting, functional mobility)
- Bed mobility or transfers (e.g., transfers to chair, toilet, commode, tub, shower, car)
- Cognitive, speech, language, or swallowing retraining
- Home and lifestyle modification (e.g., assessment and training focused on the home, job, school, or work environment)
- Pain management techniques (e.g., assessment and interventions used to relieve pain or decrease its intensity)
- Pulmonary rehabilitation (e.g., energy conservation, speech, breathing re-education, postural drainage, percussion, or vibration)
- Positioning and splinting
- Range of motion and strengthening
- Wheelchair mobility
- Ambulation

41:

Rehabilitation potential refers to the probability that therapy and medical goals are realistic and attainable based on patient's prior level of function, severity of illness or injury, and the extent of impairments.

42:

Functional assistance levels are based upon the patient's function during tasks and activities necessary to return to household mobility or ambulation. The functional assistance level required for each individual task or activity (e.g., mobility, activities of daily living (ADLs)) may vary.
The following terms are commonly used in the post-acute setting:

- Independent - Patient can safely and within a reasonable amount of time perform a task (or developmentally appropriate task) without physical or cognitive assistance or supervision

- Modified Independent - Patient performs an activity with a supportive device, adaptive equipment, and/or prosthetic or orthotic device. Additional time may be required to complete the activity and/or there are safety (risk) considerations
- Supervision - Patient performs an activity with standby or distant supervision or setup. Verbal cueing or coaxing, without physical contact or setup of items and application of orthoses may be required when patient's safety awareness is impaired
- Minimum or limited Assistance - Patient performs at least 75%(0.75) of an activity and requires some physical contact to steady, guide, or move
- Moderate or extensive Assistance - Patient performs at least 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Maximum Assistance - Patient performs 25%(0.25) to 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Total Assistance or dependence - Patient performs less than 25%(0.25) of an activity and may require total assistance for functional mobility or ADLs

43:

The intensity of rehabilitation therapies is a driver for admission to an acute inpatient rehabilitation facility. Documentation should reflect the need for required and ongoing intensive therapies. The determination of whether a patient requires and can tolerate an intensive rehabilitation program of at least 15 hours is not based on any one single piece of documentation. All clinical documentation should be evaluated by the reviewer to determine whether the patient is demonstrating the need for services and has the ability to tolerate them. Patients who do not require this level of intensive therapy should be treated at a lower level of care. Rehabilitation therapies should be reasonable and medically appropriate and should not exceed a patient's level of tolerance. If there are clinical features which preclude the patient's ability to tolerate the intensity threshold of at least 15 hours per week, secondary review would be appropriate (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Miller and Forer, Pm r 2021, 13: 535-9; Forrest et al., Medicine (Baltimore) 2019, 98: e17096).

44:

Comorbid conditions include new onset or worsening behavioral symptoms, heart failure, deep vein thrombosis, uncontrolled diabetes, immunocompromised states, malignant or end-stage disease, malnutrition, renal insufficiency or end-stage renal disease, infection, ventilator dependence, noninvasive positive pressure ventilation or respiratory insufficiency, complex wound care, and new functional impairment(s) with limitation.

45:

Ventilation includes invasive ventilation and noninvasive positive pressure ventilation (NIPPV), including continuous positive airway pressure (CPAP) and bilevel positive airway pressure (BiPAP).

46:

Notable causes for prolonged mechanical ventilation include:

- Ventilator-acquired pneumonia
- Renal failure
- Heart failure
- Exacerbation of chronic obstructive pulmonary disease (COPD)
- Chest wall disorder
- Neuromuscular diseases
- Traumatic brain injury
- Sepsis
- Complications following cardiac surgical procedures

Major contributing factors for failure to wean include (Ambrosino and Vitacca, Multidiscip Respir Med 2018, 13: 6; Davies et al., Br J Anaesth 2017, 118: 563-9):

- Increased work of breathing
- Impaired respiratory drive
- Inspiratory muscle weakness

47:

Long-Term Acute Care (LTAC) is a recognized level of care designation by the Centers for Medicare and Medicaid Services (CMS) for acute care hospitals. It is defined by federal statutes as a level of care for patients requiring an average length of stay of 25 days or more (Centers for and Medicaid Services, Fed Regist 2018, 83: 41144-784). Patients who require a short stay for treatment may be more appropriate for the subacute or skilled nursing facility (SNF) level of care.